

HOUSE BILL 2619

By Howell

AN ACT to amend Tennessee Code Annotated, Title 4;
Title 8; Title 56 and Title 71, relative to insurance.

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. Tennessee Code Annotated, Title 56, Chapter 7, is amended by adding the following as a new part:

56-7-3901.

As used in this part:

(1) "Artificial intelligence" means a machine-based system that:

(A) Can, for a given set of human-defined objectives, make predictions, recommendations, or decisions; influence real and virtual environments without significant human oversight; or that can learn from experience in an automated manner and improve such performance when exposed to data sets; or

(B) Is developed in any context, including software or physical hardware, and solves tasks requiring human-like perception, cognition, planning, learning, communication, or physical action;

(2) "Commission" means the Tennessee commission of insurance review, created pursuant to § 56-7-3902;

(3) "Covered individual" means an individual who is covered by a health benefit plan;

(4) "Covered services" means medical services for which a reimbursement is available under the enrollee's plan contract, or for which a reimbursement would be available but for the application of contractual

limitations such as deductibles, copayments, coinsurance, waiting periods, annual or lifetime maximums, frequency limitations, alternative benefits payments, or any other limitation;

(5) "Downcoding" means the adjustment of a claim submitted to a health benefit plan to a less complex or lower cost procedure code;

(6) "Fee schedule" means a list of reimbursement amounts assigned to specific codes and used by a health insurance entity pursuant to a contract between a health insurance entity and a healthcare provider to calculate payments paid to the provider for healthcare services delivered to enrollees;

(7) "Health benefit plan":

(A) Means benefits consisting of medical care, provided directly, through insurance or reimbursement, or otherwise and including items and services paid for as medical care, under any policy, certificate, or agreement offered by a health insurance entity; and

(B) Does not include policies or certificates covering only accident, credit, disability income, long-term care, hospital indemnity, medicare supplement as defined in § 1882(g)(1) of the Social Security Act (42 U.S.C. § 1395ss(g)(1)), specified disease, other limited benefit health insurance, automobile medical payment insurance, or insurance under which benefits are payable with or without regard to fault and that are statutorily required to be contained in any liability insurance policy or equivalent self-insurance;

(8) "Health insurance entity" means an entity subject to the insurance laws of this state, or subject to the jurisdiction of the commissioner of commerce and insurance, that contracts or offers to contract to provide a health benefit plan,

including, but not limited to, an insurance company, a health maintenance organization, and a nonprofit hospital and medical service corporation;

(9) "Healthcare provider" means healthcare professionals, establishments, or facilities licensed, registered, certified, or permitted pursuant to title 63 or title 68 and regulated either under the authority of the department of health or any agency, board, council, or committee attached to the department of health;

(10) "Lasering" means the practice of assigning a higher attachment point or deductible, or denying coverage, for a covered individual or group of covered individuals within the policy's coverage who have a known, high-cost medical condition or history of significant claims;

(11) "Material change" means a change to:

(A) A health benefit plan's rules, guidelines, policies, or procedures concerning payment for medical services;

(B) The general practices of the health benefit plan that decrease reimbursements paid to providers; or

(C) How a health benefit plan adjudicates and pays claims for services if the change would:

(i) Require a provider to change how the provider submits claims to the plan; or

(ii) Increase the provider's administrative expense;

(12) "Participating provider" or "provider" means a healthcare provider who provides healthcare services to a covered individual at a fee set by or at a fee subject to the approval of an insurer, health services plan, third-party administrator, or another party that contracts to provide healthcare services; and

(13) "Person" means an individual, organization, trust, foundation, group, association, partnership, limited liability company, corporation, society, or any other entity.

56-7-3902.

(a) There is created the Tennessee commission of insurance review for the purpose of reviewing complaints related to and enforcing the requirements of this part.

The commission consists of seven (7) members as follows:

(1) The commissioner of health, or the commissioner's designee, who serves as chair;

(2) One (1) member, appointed by the speaker of the house of representatives, with no less than five (5) years' experience as a healthcare professional;

(3) One (1) member, appointed by the speaker of the senate, with no less than five (5) years' experience in the health insurance industry;

(4) Two (2) members of the house of representatives, appointed by the speaker of the house of representatives; and

(5) Two (2) members of the senate, appointed by the speaker of the senate.

(b)

(1) To stagger the terms of the newly appointed commission members, the appointing authorities shall make initial appointments as follows:

(A) The member appointed under subdivision (a)(2) is appointed to a three-year term, which expires on June 30, 2029; and

(B) The member appointed under subdivision (a)(3) is appointed to a four-year term, which expires on June 30, 2030.

(2) Following the expiration of members' initial terms as prescribed in subdivision (b)(1), all appointments to the commission shall be for terms of four (4) years and shall begin on July 1 and expire on June 30, four (4) years thereafter.

(3) Legislative members of the commission appointed under subdivisions (a)(4) and (5) serve terms of two (2) years and may be reappointed for one (1) additional two-year term, with both terms running commensurate with their service as members of the general assembly.

(4) Members appointed under subdivisions (a)(2)-(5) continue to serve until a successor has been appointed.

(5) Vacancies occurring for members appointed under subdivisions (a)(2)-(5), other than through the expiration of terms, are filled for the remainder of the term of the member being replaced.

(6) Commission members described in subdivisions (a)(2) and (3) are eligible for reappointment, but do not succeed themselves automatically.

(7) Appointments to the commission for the remainder of an unexpired term and reappointments must be made in the same manner as under subdivisions (a)(2)-(5).

(c) Each member of the commission described in subdivisions (a)(2)-(5) is entitled to receive reimbursement for the member's traveling and other necessary expenses actually incurred while engaged in the performance of any official duties when so authorized by the commission, but such expenses must be made in accordance with the comprehensive state travel regulations duly promulgated by the commissioner of finance and administration and approved by the attorney general and reporter.

(d) A majority of the commission constitutes a quorum, and the concurrence of a majority of those present and voting on a matter is required for a determination of matters within the commission's jurisdiction.

(e) The commission is to be funded from the general fund as appropriated annually in the general appropriations act, with such funding to be included for, but not limited to, the commission's administrative costs, hiring of staff necessary to carry out the commission's duties, and for costs of any programs in support of the purposes for which the commission is organized. The commission may also be financed through private funding and through public and private grants.

56-7-3903.

(a) The commission has the power and authority to:

(1) Adopt, modify, repeal, promulgate, and enforce rules for:

(A) The conduct of the affairs of the commission;

(B) Enforcement of the commission's orders; and

(C) Otherwise effectuating this part;

(2) Issue subpoenas requiring the attendance of witnesses and production of such evidence as requested, administer oaths, and take such testimony as the commission deems necessary in fulfilling its purpose. If a person refuses to obey a subpoena issued by the commission under this part, then the chancery court of Wilson County has jurisdiction upon application of the commission to issue an order requiring the person to appear and testify or produce evidence as the case may require, and a failure to obey the order of the court may be punished by the court as contempt;

(3) Offer advisory technical assistance to a health insurance entity or participating provider regarding appropriate claim or code review practices, to the extent it may be competently offered;

(4) Order remedial actions regarding violations of this part, including health insurance entity coding or claim practices; and

(5) Exercise all the powers and take all the actions necessary, proper, or convenient for the accomplishment of the purposes enumerated in this part.

(b)

(1) In addition to the powers described under subsection (a), the commission shall review and conduct informal hearings of complaints submitted to it regarding violations of this part.

(2) A complainant to the Tennessee commission of insurance review shall first appeal or make a complaint through the standard claim appeal process, or utilize any other available remedy offered by the health insurance entity, prior to seeking an informal hearing before the commission.

(3) The commission shall consider the reasonableness of the health insurance entity's claim or code review practices and policies, use of such review practices in connection with the submitted complaint, and other evidence presented during the hearing, if applicable, in making the commission's decision.

(4) The request for an informal hearing under this subsection (b) must be received within thirty (30) days of the final adverse decision of the health insurance entity.

(5) Appellate review of the commission's decisions is governed by the Uniform Administrative Procedures Act, compiled in title 4, chapter 5. The

Wilson County chancery court has jurisdiction over judicial review of the commission's decisions.

(6) In the conduct of an informal hearing upon complaint, the commission may receive affidavit evidence, in addition to transcripts and other evidence of actions by the health insurance entity or participating provider, and may render its decision on the basis of that evidence or, if the commission deems an open hearing appropriate, may order the interested parties be notified of the date, time, and place that such hearing will be held.

(7) The commission may:

(A) Order such remedial actions as necessary to address a participating provider's complaint, including the exercise of any authority conferred upon the commission by this part; and

(B) Request that the attorney general and reporter bring an action in a court of competent jurisdiction in the name of the state against an individual or entity for violations of this part.

(8) Commission staff may consolidate complaints that raise substantially similar issues against the same health insurance entity to be heard together before the commission.

(c) The commission may participate by electronic or other means of communication for the benefit of the public and the commission in connection with any meeting authorized by law.

56-7-3904.

(a) A health insurance entity shall not offer or maintain in this state a health benefit plan that based on the participating provider's contracted fee for covered services

uses downcoding in a manner that prevents the provider from collecting the fee for actual services performed either from the health benefit plan or the patient.

(b)

(1) Except as provided in subdivision (b)(2), a health insurance entity shall not offer or maintain in this state a health benefit plan that allows the health insurance entity to:

(A) Make material changes to the health benefit plan, including by changing a provider's fee schedule, a provider manual, or a reimbursement rule or policy, without:

(i) Providing at least sixty (60) days' notice and an opportunity to negotiate the changes to the affected provider; and

(ii) Allowing an affected provider to terminate the provider's contract with the health insurance entity without penalty if the health insurance entity and the affected provider do not come to an agreement regarding the proposed changes to the health benefit plan; or

(B) Otherwise unilaterally make changes to the contract between the health insurance entity and a provider.

(2) Subdivision (b)(1) does not apply to the following changes to a health benefit plan:

(A) Any change in a provider's fee schedule due to a change effected by the federal or state government to its healthcare fee schedule, if the provider and health insurance carrier have previously agreed that the provider's fee schedule is based on a percentage or some other

formula of a current government healthcare fee schedule, such as medicare;

(B) Any change in a provider's reimbursement for drugs, immunizations, injectables, supplies, or devices if the provider and health insurance carrier or pharmacy benefits manager, as defined by § 56-7-3102, have previously agreed that any reimbursement for drugs, immunizations, injectables, supplies, or devices will be based on a percentage, or some other formula, of a price index not established by the health insurance carrier, such as the average wholesale price or average sales price;

(C) Any changes in the provider's reimbursement for drugs, immunizations, injectables, supplies, or devices if the provider and the health insurance carrier or pharmacy benefits manager, as defined in § 56-7-3102, have previously agreed to any reimbursement for drugs, immunizations, injectables, supplies, or devices in accordance with § 56-7-3104 and based upon maximum allowable cost pricing as regulated by §§ 56-7-3101 and 56-7-3106;

(D) Any change to Current Procedural Terminology (CPT) codes, Healthcare Common Procedure Coding System (HCPCS) codes, International Statistical Classification of Disease and Related Health Problems (ICD) Codes, or other coding sets recognized or used by the centers for medicare and medicaid services (CMS) that a health insurance carrier utilized in creating a provider's fee schedule;

(E) Any change to revenue codes as established by the National Uniform Billing Committee (NUBC);

(F) Any changes in a provider's fee schedule due to one (1) or more of the following if previously agreed to in a provider's agreement with a health insurance carrier:

(i) Payments made to the healthcare provider by the health insurance carrier or payments made to the health insurance carrier by the provider that are based on values or quality measures explicitly described in the written agreement between the provider and the health insurance carrier intended to improve care provided to the health insurance carrier's members;

(ii) Escalator or de-escalator clauses;

(iii) Provisions that require adjustments to payment due to population health management performance or results; or

(iv) Any arrangements, initiatives, or value-based payments relating to or resulting from the implementation or operation of the Tennessee Health Care Innovation Initiative or any successor state program applicable to provider agreements covered by this section; or

(G) Changes in standard codes and guidelines developed by the American Medical Association or a similar organization.

56-7-3905.

A health insurance entity shall not:

(1) Send an enrollee's medical information or history to a person located outside of the United States for any purpose, including the review of claims submitted by a healthcare provider; or

(2) Enter into or renew a contract with a person that authorizes or requires the health insurance entity to send an enrollee's medical information or history to a person located outside of the United States for any purpose.

56-7-3906.

A health insurance entity shall ensure that coding or claim reviews that are conducted using artificial intelligence are reviewed, approved, or sent for further review by a human healthcare provider reviewer prior to the health insurance entity sending a determination to the participating provider who submitted the claim or the covered individual for whom the services underlying the submitted claim were performed. The healthcare provider who reviews and approves a coding or claim review conducted by artificial intelligence must have relevant experience in the healthcare field to which the code or claim being reviewed relates.

56-7-3907.

A health insurance entity shall not remove an enrollee from a group or group plan if the enrollee was covered the prior year in the same group or group plan, except:

- (1) For nonpayment of the required premiums by the policyholder or contract holder;
- (2) For fraud or misrepresentation of the policyholder or contract holder or, with respect to coverage of individual enrollees, the enrollee or the enrollee's representative; or
- (3) When the number of enrollees covered under the plan is fewer than the number of insureds or percentage of enrollees required by participation requirements under the plan, if applicable.

56-7-3908.

A health insurance entity shall not use lasering when underwriting a stop-loss plan for a health benefit plan offered in this state.

56-7-3909.

(a) On or after July 1, 2026, a person shall not directly or indirectly own, operate, or control the whole or any part of a healthcare provider in this state, including through investment interests, while the person also directly or indirectly owns, operates, or controls the whole or any part of a health insurance entity, including through investment interests.

(b) If a person directly or indirectly owns, operates, or controls the whole or any part of both a healthcare provider and a health insurance entity prior to July 1, 2026, then the person shall:

(1) Not require a covered individual to utilize a healthcare provider over which the person has such direct or indirect ownership or control; provided, that this subsection (b) does not prohibit the inclusion of such healthcare entity or provider as an available option to the covered individual if the covered individual is given additional choices of similar quality; and

(2) If the person is negotiating with the state to provide a plan managed by the health care finance and administration division of the department of finance and administration or a successor division or department, or the group insurance plans offered under title 8, chapter 27, disclose such ownership, operation, or control.

56-7-3910.

(a) A participating provider or covered individual who believes a health insurance entity has violated this part may submit a complaint to the Tennessee commission of insurance review.

(b)

(1) In addition to the remedy described in subsection (a), upon request by the Tennessee commission of insurance review or receipt by the attorney general and reporter of other relevant information, the attorney general and reporter may bring an action in a court of competent jurisdiction in the name of the state against an individual or entity relative to a violation of this part. In a cause of action brought by the attorney general and reporter pursuant to this subdivision (b)(1), the court may:

(A) Issue orders and injunctions to restrain and prevent violations of this section;

(B) Impose compensatory and punitive damages; and

(C) Order reasonable attorney fees and investigative costs be paid by the violator to the state.

(2) An action brought by the attorney general and reporter pursuant to subdivision (b)(1) may also include other causes of action.

56-7-3911.

Notwithstanding another law to the contrary, including § 56-7-1010, and except as provided under § 56-7-3910(b), the commission has exclusive enforcement authority of this part.

56-7-3912.

(a) The department of commerce and insurance and its employees shall fully cooperate with the commission in the execution of this part by timely providing requested information and sharing relevant documents and data.

(b) The transfer or sharing of documents, data, records, or other information pursuant to this section does not waive an applicable privilege from disclosure.

SECTION 2. Tennessee Code Annotated, Section 4-29-249(a), is amended by adding the following as a new subdivision:

() Tennessee commission of insurance review, created by § 56-7-3902;

SECTION 3. Tennessee Code Annotated, Section 56-7-1013(c), is amended by deleting subdivisions (1) and (3).

SECTION 4. Tennessee Code Annotated, Section 56-7-1013, is amended by deleting subsection (g).

SECTION 5. Tennessee Code Annotated, Section 56-7-3302, is amended by deleting the section.

SECTION 6. Tennessee Code Annotated, Title 56, Chapter 7, Part 10, is amended by adding the following as a new section:

(a) If a covered service is within the scope of practice of a provider with a particular license classification and a health insurance entity provides reimbursement for that service when performed by a provider with a different license classification, the health insurance entity shall not deny reimbursement or pay a different reimbursement rate to a provider with a different license classification, regardless of the billing modifier or coding designation used to identify the provider of the service.

(b) A health insurance entity shall not establish or apply a different fee schedule, reimbursement methodology, conversion factor, unit valuation, payment adjustment, site-of-service differential, or code restriction, solely on the basis of the healthcare provider's licensure classification being different from the classification of a medical doctor or osteopathic physician. This subsection (b) regulates health insurance entity practices as part of the business of insurance and does not apply to any plan or arrangement governed by the Employee Retirement Income Security Act of 1974 (ERISA) (29 U.S.C. § 1001 et seq.), including, but not limited to, self-funded or self-

insured employer-sponsored health plans. This subsection (b) is intended to fall within the scope of ERISA § 514(b)(2)(A) as a law that regulates insurance.

(c) A health insurance entity shall not implement or enforce reimbursement provisions that are less favorable for healthcare providers licensed under a different profession when performing the same or substantially similar services within their lawful scope of practice.

(d) This section does not expand a provider's scope of practice or require a health insurance entity to contract with a particular provider.

(e) A health insurance entity shall not reduce reimbursement rates for physicians or other providers for the purpose or effect of avoiding compliance with the requirements of this section.

SECTION 7. All rules and regulations issued or promulgated by the department of commerce and insurance that relate to the functions, duties, and responsibilities of the Tennessee commission of insurance review are transferred to the commission, to be administered and enforced by the commission on the effective date of this act.

SECTION 8. This act takes effect July 1, 2026, the public welfare requiring it, and applies to conduct occurring on or after that date.